جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ليلي محمد محمد محمود
المحافظة : القاهره **الحي / المدينة :** المرج **العنوان :** //
رقم البطاقة : 28610140102421 **رقم التليفون :** 01100646024 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 14-10-1986 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه [11112] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ليلى محمد محمد محمود
National ID: 28610140102421

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the brain and orbits

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
مسعودى
وشين على اعلى



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي انترناشونال مصر

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 14/10/2025 G

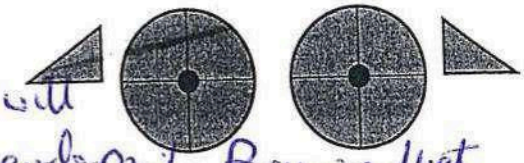
Name	سلمى محمد	MRN	90910CE
		ID	CA71121CECI
Age	38 y.	Phone	01100646024
Menopausal status	pre-m	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity			

Ciinical Data

BX:

ER:	PR:	HER 2:	Ki67:
+ve 6/8	+ve 6/8	3+	

R PS 1 L



Sonomammography: Rt Breast cancer with Newly developed Brain Mets

Mets Workup: Hist:- pre-M Rt Breast cT3, N1, M0 Rec NACT 4 AC, 4 Taxotere, 5 HP on 25/9/2024 → Rt SSM + ALND

TNM

Recommendations: cR 4 pT0 4 pN0 0/16 St. Nolvadex f/u 10/2024, endocrinology clinic on 3/8/2025 Recently Received HRI Brain with Cont on 5/9/2025 left occipital intraaxial SOL 3 cm x 2.5 + Medical Vasogenic

MDT OUTCOME & PLAN CT 8/2025 f/u MDT 1) Neurosurgery consultation for possible surgical excision 2) Stereotactic Radiation 3) Tykerb / Xeloda 14/11 Rebiopsy f/u MDT

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
	Dr. Hazem 14/9/2025				

Name: ليلي محمد محمد محمود	
MR. : 1106	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

CT CAP FREE MRI POST-OPRETIVE FREE AT operative TO CONTINUE XELODA TYKERB

Pt underwent excisional biopsy from brain SOL: metastatic carcinoma likely of breast origin (for IHC GAT3, ER, CK). She received WCI ? as she has also skull lesion. Now she is on Xeloda-Tykerb + Zoladex and will do the IHC on brain lesion biopsy.

Pt came today to counsel us on getting pregnant & resuming Zoladex for fertility preservation. I counselled the pt on the risk of stopping active ttt to get pregnant, and will start non-hormonal contraception & keep under ttt for the current time. Results of IHC on brain specimen are pending.

MBC to brain meatstectomy pathology ER5/8 HER2 + PR 0/8. ON TYKERB/XELODA LAST IMAGINE 2/2026 FREE CO OF SEVER GASTRTIS FOR SUPPORTIVE TTT

LAB NORMAL FOR XELODA TODAY TYKERB NEXT WEEK WITHOUT LAB

TODAY FOR TYKERB, XELODA NEXT VISIT AFTER 21 DAYS

TODAY FOR XELODA +TYKERP TODAY ARRANGED FOR UPPER GI

Upper GI endoscopy 3/6/2026 revealed: Gastroesophageal reflux disease (LA grade B) incompetent cardia erosive gastritis erosive bulb duodenitis for 1- MRI brain and bone scan 2- for tykerb today 3- supportive ttt

MRI brain free only postoperative changes, bone scan free to continue on XELODA TYKERB for zoladex today

today for xeloda 5*1*14 ,+ supp ttt after labs bon scan 20-6-2026 >> considering the last bone scan report dated 22/12/2025, the current scan revealed still noted skull lesion as described warranting CT correlation to clarify its nature. No new developed bone lesion could be depicted. Follow up bone scan is advised to monitor the therapeutic response CT CAP >> R

Premenopausal female, known case of R breast ca, cT3N1M0, HER2+. NAC: 4 AC → 4 Docetaxel + 5 Trastuzumab. 09/2024: R skin-sparing mastectomy + ALND → pCR: ypT0N0 (0/16 LN). 10/2024: Started adjuvant Tamoxifen. 08/2025: Completed adjuvant Trastuzumab. 10/2025: New-onset headache → MRI brain: L occipital intra-axial SOL, 3.5 × 2.5 cm, with moderate vasogenic edema. Excision: Pathology → metastatic breast carcinoma; IHC: ER 5/8, PR 0/8, HER2 FISH 3+. Post-op RT: WBRT 20 Gy/5 Fx + boost 10 Gy/5 Fx. Subsequently started Capecitabine + lapatinib. Latest CT CAP 5/8/2026: No

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : محمد رضا أبو اليزيد كيلاني

يعتمد ,

pulmonary or hepatic deposits. for xeloda x5x14 + lapatinib 1x5x30, labs accepted, no new complaints except for fatigue caused by capecitabine

known breast cancer with brain metastatsis on tramadol 50 mg / 12h will continue the same regimen.

The patient has completed 5 cycles of Xeloda and 5 cycles of lapatinib.

GUSTAVE/
ROUSSY

Patient Name	: الأستاذه / ليلى محمد محمد محمود	Request No.	: 13005421
Patient No.	: 28610140102421	Request Date	: 11/03/2026
Path No.	: SR-363/26	Verification Date	: 15/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 39 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left brain occipital SOL in patient with history of left breast cancer.
Nature of Specimen: Referred blocks & slides (Biopsy).

Gross Examination:
- Four referred blocks & four referred (H&E) stained slides were received labelled by (16133-25).

Microscopic Examination:
- Histopathological examination reveals portions of tumor formed of malignant epithelial cells showing moderate to marked nuclear pleomorphism and eosinophilic cytoplasm, arranged in clusters & nests with moderate tendency to tubular formation amid desmoplastic stroma. Comedo type necrosis is noted.

Diagnosis:
Left brain occipital SOL, Referred blocks & slides (Biopsy):
Findings are consistent with metastatic carcinoma of breast origin.
- Case for further (GATA-3, ER, PR & HER2).

Signature
Dr. Walaa Ibrahem Ghoneim.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY

Patient Name	: الأستاذه / ليلى محمد محمد محمود	Request No.	: 13005421
Patient No.	: 28610140102421	Request Date	: 11/03/2026
Path No.	: SR-363/26	Verification Date	: 15/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 39 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left brain occipital SOL in patient with history of left breast cancer.
Nature of Specimen: Referred blocks & slides (Biopsy).

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER clone (1E2), PR clone (SP1), Her2 clone (4B5), GATA3 (L50-823).

Results:
- IPT (Positive control showed appropriate reactivity):
- GATA3: Diffuse strong positive staining in tumor cells.
- ER: Weak nuclear staining in 60% of tumor cells (score 5/8).
- PR: Negative nuclear staining in tumor cells (score 0/8).
- Her2: Strong complete membrane staining in more than 10% of tumor cells (score 3+).

Conclusion:
Left brain occipital SOL, Referred blocks & slides (Biopsy):
Findings are consistent with metastatic carcinoma of breast origin.
IPT:
- GATA3: Strong positive staining in tumor cells.
- ER: Positive (score 5/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2,

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / ليلى محمد محمد محمود	Request No.	13005421
Patient No.	28610140102421	Request Date	11/03/2026
Path No	SR-363/26	Verification Date	15/03/2026
Sex	Female	Financial Category	جوستاف روسي - نفقة الدولة
Age	39 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

intermediate; and 3, strong). Total score from 0 to 8.
Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

alfa
LABS

Collection Date: 07/11/2025
Review Date: 11/11/2025

Referring Doctor: أ.د احمد حسن الحاملولي

Patient Name: الاستاذة/ ليلي محمد محمد محمود
Patient No.: 164-20579
Accession No.: 16425118344
Age: 39Y

PATHOLOGY UNIT

Path. No.: (16133-25).

Clinical Data & Nature of the specimen:
Left brain occipital S.O.L; history of breast cancer.

Gross Examination:
Multiple irregular grayish tan tissue pieces collectively measured 6x6 cm with soft to rubbery grayish white cut section.

Grossing by W. Elsaid

Microscopic Examination:
Sections showed malignant growth disposed in nests and sheets of cells with moderate nuclear anaplasia and focal comedo like necrosis.

Diagnosis:
Left brain occipital S.O.L; history of breast cancer:-

*Features consistent with metastatic carcinoma likely of breast origin for Immunophenotyping confirmation (GATA3, ER, CK).
*Correlate with the clinical, radiologic and PET-CT findings.

Dr. Eman Ali, FEBP

Prof. Dr. Dalia M. Abouelfadl, MD

Reviewed By:

Pt. Branch: وحدات الرئيسي

Printed by: 37710خالد جمعة روبي

AMG ALFA MEDICAL GROUP A MEMBER OF ALFA MEDICAL GROUP
معامل الفا
16101

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Name : Laila Mohamed Mohamed Mahmoud	Date : 17-02-2026 10:48
Branch : Portable	Medical No. : GRI-94173
Ref.Physician Prof : ct cap+c	Ins.Company :

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

In comparison to previous report dated 20/8/2025 current study reveals:

- Right breast tumour resection with no soft tissue lesion by CT criteria.
- Right middle lobe atelectatic bands.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

- ***Essentially normal CT study of the chest.***

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL EGYPT

Name : Laila Mohamed Mohamed Mahmoud	Date : 17-02-2026 10:48
Branch : Portable	Medical No. : GRI-94173
Ref.Physician Prof : ct cap+c	Ins.Company :

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with low homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

No time interval changes from last CT chest and abdomen.
Fatty liver.

Dr Mai Mohamed MD

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Laila Mohamed Mohamed Mahmoud	Date : 18/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 39 Year(s)
Medical ID: 959/2024	Pat. ID: 94173

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

FINDINGS:

Known case of breast CA underwent conservative breast surgery. Compared to the previous study dated 20/8/2025, the current study showed:

(I) Chest:

- Still noted few bilateral scattered thin atelectatic bands.
- Still seen right middle lobar reticulations of post radiation therapy sequel.
- The rest of both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Still noted increased breast density likely postoperative changes, for sono-mammographic correlation.

(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density with no focal lesions or dilated IHBRs.
- Patent and homogenously enhancing portal vein.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Unremarkable other pelvic organs.
- Few subcentimetric para-aortic and mesenteric lymph nodes.
- No ascites.
- No worrisome lytic or sclerotic bone lesions.

OPINION:

- **Stationary course since last study.**

Dr. Sherif Wagdy

19.10.2025	تاريخ الفحص	690333	حجز رقم
01100646024	التليفون	ا/ ليلى محمد محمد محمود محمد	313642 المريض
MRI.Brain&MRS &contrast	الفحص	39 y 0 M 15 D السن	Female النوع

Examination:

- MRI of the brain With Contrast.

Technical data:

- Axial T1, T2& FLAIR.
- Sagittal T1
- Post contrast axial , sagittal and coronal T1 WI.

Findings:

- **Evidence of left occipital well-defined intra axial mass lesion seen in the sub cortical region with surrounded Grade II vasogenic edema and sulcal effacement with mild compression the occipital horn of the ipsilateral lateral ventricle, eliciting abnormal signal iso in T1 , T2 and FLAIR shows marginal post contrast enhancement with central areas of necrosis measuring 3.5 X2.5 X3cm as (Metastatic deposit with cystic degeneration)**
- The rest brain parenchyma shows normal signal pattern with no cerebral parenchymal areas of abnormal signal intensity.
- Normal size and shape of the ventricular system, with no mass effect or midline shift.
- Normal appearance of the extra-axial C.S.F. spaces, including supra-seller cisterns and cerebello-pontine angle cisterns.
- No evidences intra-cranial sizable vascular malformations detected

Best Regards,
DR KHALED GAMAL, MD

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MR Spectroscopy:

Single voxel with short, intermediate and long TE in the lesion.
Single voxel with intermediate TE in the corresponding normal region.
Multivoxel with long TE in the lesion.

- **The choline level** is elevated 28 in the lesion comparable to 3.2 in the normal brain denoting increased rate of cell membrane turn over.
- **The NAA level** is decreased from 3.9 in the normal brain to 3.4 in the lesion denoting either destruction or replacement of the neural tissue by the pathological process.
- **The creatine level** is decreased from 7.04 in the normal brain to 5.7 in the lesion coincided with increased metabolic demands.
- **The lactate peak:** A peak is noted at 1.33 ppm and inverted at the intermediate TE denoting anaerobic metabolism within the lesion.
- **The myoinositol peak:** not present
- **Increased Cho/Cr ratio and Increased Cho/NAA ratio**

OPINION:

- **The current MR diffusion, MR perfusion and MR spectroscopy findings are in favour of Metastatic deposit with cystic degeneration**

DR/KHALED GAMAL ,MD

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أ.د. أشرف عنايت د. ممدوح فايز
د. أيهاب عادل د. ماجد سليم
أ.د. محمود الشامي إستشارى الأشعة
دكتوراه الأشعة التشخيصية والتداخلية

Name: Mrs. Laila Mohamed Mohamed
Ref.Prof.Dr.: Ahmed Hassan Elhamoly
Date: 08/02/2026
Acc No: 831738

MRI study of the brain with contrast

Techniques:

- *Variable echo pulse sequences were taken through the brain in different planes pre and post contrast.*

Finding:

*** Status a known case of metastasizing breast cancer subjection to surgical excision with chemo and radiotherapy, no previous available films for comparison however the current study has revealed:*



- *The obtained D WIs revealed no evidence of recent ischemic infarct.*
- *Left parietal focal cystic encephalomalacic gliotic changes with hemosiderin deposition and overlying left parietal craniotomy. No signs of significant post contrast enhancement.*
- *Few bifrontal subcortical tiny non specific white matter foci of altered FLAIR signal likely vasculopathic in nature.*
- *The ventricular system is within normal limits.*
- *There is no obvious intra axial mass lesion, signal of blood degradation or extra axial fluid collection.*
- *The sellar and suprasellar structures appear intact.*
- *The posterior fossa structures including the brain stem appear intact.*

Opinion:

*** A known case of metastasizing breast cancer subjection to surgical excision showing:*

- *No evidence of recent ischemic infarct by D WIs.*
- *Left parietal focal cystic encephalomalacic gliotic changes with hemosideri deposition.*
- *No signs of sizeable residual/recurrent enhancing lesion by current MRI criteria fo guided follow up.*
- *Few bifrontal vasculopathic white matter foci.*

Best regards
Dr. Marwa Sayed, MD

Name : Leila Mohammed Mohammed Mahmoud
Branch : Dokki

Date : 22-12-2025 12:39
Medical No. : 279166

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History

- A 39-year-old lady, with history of right breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- Enhanced tracer uptake is seen at the left high parietal region of the skull.
- No other abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **Enhanced tracer uptake at the left high parietal region of the skull, for correlation with clinical/radiological data.**
- **The rest of skeleton is free from metastases.**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient's Name:	**Laila Mohammed Mohammed Mahmoud**
Date:	**2026-08-05**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Status post right breast intervention.
- Few bilateral scattered atelectatic bands.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing diffuse low parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ **No pulmonary or hepatic metastatic lesions.**

Dr.sherif Wagdy